جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ساميه محمد مدبولي عمر
المحافظة : القاهره **الحي / المدينة :** المقطم **العنوان :** مسكن رولان
رقم البطاقة : 25410280103061 **رقم التليفون :** 01001580439 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 28-10-1954 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي مثبط CDK4/6 و فلفسترانت [14469] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ساميه محمد مديولي عمر
National ID: 25410280103061

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

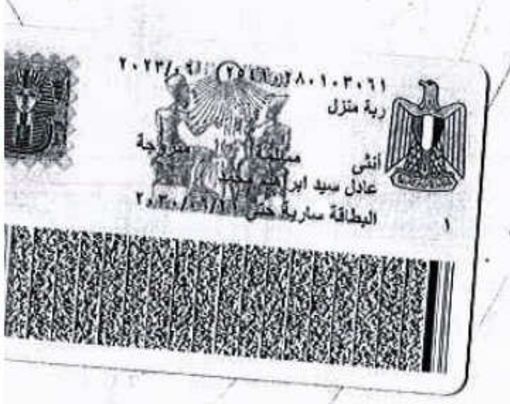
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي CDK4/6 inhibitor and fulvestrant

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	: الأستاذه / ساميه محمد مدبولى عمر	Request No.	: 13001838
Patient No.	: 25410280103061	Request Date	: 19/03/2022
Path No.	: 771/2022	Verification Date	: 09/04/2022
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 65 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast lesion
Nature of the specimen:Tru cut biopsy
Gross Examination:
Multiple greyish white tissue cores up to 1.5 cm , totally submitted

Microscopic examination:
- Histological type:Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma insitu : Focal

Diagnosis:
Right breast lesion, tru cut biopsy:
Invasive duct carcinoma grade II
IPT:
ER: Strong nuclear reaction in about 90% of tumour cells. Positive, score (8/8).
PR: Strong nuclear reaction in about 90% of tumour cells. Positive, score (8/8).
Her-2: Weak incomplete membrane reaction in more than 10% of tumour cells. Negative, score 1+
Ki67: 25-30% nuclear reaction.

Prof. Dr. Gamal Abd-El Aty
Dr. Esraa Ibrahim
Dr. Abdulrahman Ibrahim

عبد الرحمن ابراهيم
9/4

Verified by :
abdelrahman ibrahim

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - شارع الكورنيش - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Name: سامیه محمد مدبولي	
MR. : 4684	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		**رقم طبي:**	

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 71-year-old female with a history of right-sided breast cancer treated with right mastectomy and adjuvant chemotherapy, followed by radiation therapy to the breast and regional nodes. She has concurrent osteoarthritic changes of the acromioclavicular joint, supraspinatus tendinopathy, a partial thickness tear, and subcoracoid bursitis. Imaging of the cervical spine in October 2025 revealed vertebral collapse with abnormal bone marrow signal at C4, C5, and C6, along with disc bulge. A recent breast ultrasound performed on 8 March 2026 demonstrated a free femoral calcium deposit measuring 1 × 1 × 90 mm, described as "one alpha." The patient is currently on a regimen of femara calcium 1 × 1 × 90 mg, with an additional calcium 1 × 1 × 90 mg supplement. She has declined a bone scan and is scheduled for her next clinic visit on 26 August 2026. The patient remains under observation for bone health and oncologic surveillance.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

[[DOCTYPE:OTHER_DETAILED_MEDICAL_REPORT]]

Patient Name	: الأستاذه / ساميه محمد مدبولى عمر	Request No.	: 13001838
Patient No.	: 25410280103061	Request Date	: 19/03/2022
Path No.	: 771/2022	Verification Date	: 06/04/2022
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 65 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:
Right breast lesion
Nature of the specimen:Tru cut biopsy
Gross Examination:
Multiple greyish white tissue cores up to 1.5 cm , totally submitted

Microscopic examination:
- Histological type:Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma insitu : Focal

Diagnosis:
Right breast lesion , tru cut biopsy
Invasive duct carcinoma , grade II
For IPT

Prof.Gamal Abdelatty
Dr.Esraa Ibrahim

Verified by :
dr.esraa magdy

Room No. (4) First Floor Building No. 937 Nile Corniche Misr El Qadema Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

مستشفى أورام دار السلام
(مرسل)

Name: Samya Mohamed **Date: 18-06-2022**

NECK VEINS DOPPLER REPORT

Right side:

- ❑ The internal jugular, innominate, & subclavian vein are patent, compressible with normal color-flow pattern and monophasic Doppler spectral waveform, exhibiting respiratory phasicity and preserved atrial pulsations.

Left side:

- ❑ The internal jugular, innominate, & subclavian vein are patent, compressible with normal color-flow pattern and monophasic Doppler spectral waveform, exhibiting respiratory phasicity and preserved atrial pulsations.

CONCLUSION

- No central venous thrombosis.

Dr. Sherif Mostafa, M. Sc.

Ultrasound
[[DOCTYPE:ULTRASOUND]]

وزارة الصحة
أمانة المراكز الطبية التخصصة
مستشفى معهد ناصر
Nasser Institute Hospital
معهد ناصر
مركز الأورام
وحدة الطب النووي

NASSER INSTITUTE
ONCOLOGY CENTER
NUCLEAR MEDICINE UNIT

Patient Name: Samya Mohamed Madbouly. *Date: 19/7/2022*

^{99m}Tc-^{99m}Tc-MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- Degenerative changes are noticed over Dorso-lumbar vertebrae.
- No definite hot spot that accounts for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions.

Thanks for the referral

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

1351Kornlesh El Nile - Aghakhan - Shoubra - Cairo Tel: 22039164 - 22039356 Fax: 220 9 236

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Name : Samia Mohammed **Date :** 10/04/2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. *Chest:*
- Right breast asymmetrical density.
- Right axillary level I and II enlarged lymph nodes the largest measures 2.3 X 1.4 cm.
- Left lower lung lobe calcified nodule mostly representing post granulomatous sequel.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi.
- Azygous lobe and fissure.

2. *Abdomen and pelvis:*
- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. *The scanned skeleton:*

- Bilateral acetabular sclerotic foci mostly representing bone islands.
- L1 lytic lesion measuring 1.5 cm with vertical striation likely osseos hemangioma , no collapse or soft tissue component.

OPINION:

- ***Reported case of breast carcinoma.***
- ***No definite hepatic or pulmonary metastatic deposits.***

Dr.Mohammed Mohsen, MSc.

Dr. M. Gomaa, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

[DOCTYPE:BONE_SCAN]
Bone Scan

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Samia Mohammed Madbouli	Date : 04-09-2026 09:18
Branch : Maadi	Medical No. : 454158
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستشفيات

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 72-year-old female with left breast carcinoma, status post mastectomy. Presents with generalized bony aches. Referred for follow-up skeletal evaluation.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Increased tracer uptake is noted at the region of cervical spine, for clinical/radiological correlation.
- A small focal area of uptake is noted at the right aspect of L3, likely facet joint arthropathy.
- Mild symmetrical uptake is noted in both knees, slightly more pronounced on the right, consistent with degenerative changes.
- The remainder of the axial and appendicular skeleton shows physiological tracer distribution without focal osteoblastic lesions.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- No scintigraphic evidence of skeletal metastases.
- Clinical correlation and targeted imaging of the indeterminate uptake at cervical spine is advised if symptoms persist.
- Follow up imaging as clinically indicated.

Dr. Alaa Nagi MD

Nuclear Medicine

NILE DIAGNOSTIC CENTER

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Samia Mohamed Madboli Omar	Date : 07/04/2025
Exam : right upper limb venous doppler	Age : 70 Year(s)
Medical ID: 942/2022	Pat. ID: 13798

Duplex Examination of the arterial&venous system of the right upper limb revealed: -

Arterial system:

- *Patent arterial system of the examined upper limb including subclavian, axillary, brachial, radial & ulnar arteries with normal intimal thickness.*
- *Preserved arterial triphasic waveform configuration with average flow velocities of the examined arterial tree.*
- *No evidence of arterial stenosis or occlusion seen.*

Venous system:

- *The subclavian, axillary, radial & ulnar veins of the examined upper limb all are patent, totally compressible with normal color filling & preserved respiratory variation.*
- *No evidence of D.V.T*
- *patent basilic and cephalic vein along their whole course, no evidence of superficial thrombophlebitis at time of scan*

OPINION: -

No evidence of D.V.T or superficial thrombophlebitis at time of scan

No evidence of arterial stenosis or occlusion seen

Signature Dr . Nermien Ramadan
07/04/2025 12:16 PM

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Samia Mohamed Madboli Omar	Date : 03/02/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 69 Year(s)
Medical ID: 942/2022	

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM in

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- The left breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Dr.Yomna Tarek,M.Sc.

Not Approved

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Samia Mohamed Madboli Omar	Date : 08/03/2025
Exam : MAMMOGRAPHY BOTH	Age : 70 Year(s)
Medical ID: 942/2022	Pat. ID: 13798

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the left breast (ACR B)
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- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

08/03/2025 12:55 PM

جوستاف روسي

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

09/20/2026

MDT FORM

Name:	Samia Mohamed Madbouly *\	**MRN:**	4684
Age:	72 Y	**ID:**	25410280103061
Menopausal Sta		**Phone:**	01001580439
NRS SCORE:	PS ECOG 1-2	**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	free	**Reason Of MDT:**	met

Clinical Data:

BX: IDC Gli

ER: 8/8 **PR:** 8/8 **HER2:** -ve **KI67:** 40%

Sonomammography:

Mets Workup:

History:
female pt 70 yrs ER , PR +ve , her2 - ve
underwent Rt MRM + SLNB (5-2022) >> pT2 N3 adj tXT , endoxan >> PORT >> on AI since 2023 till now
recently : Bone scan (8-9-2026) >> free

cervical MRI (19-8-2026) >> C2 C3 C4 vertebral bodies with cord identation at C4 , intraspinal soft tissue mass due to neoplasrtic lesion

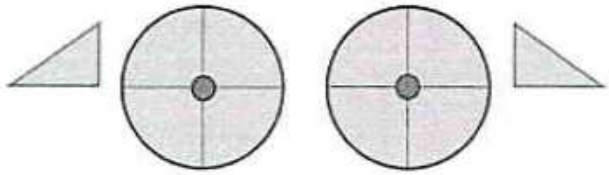
for urgent palliative RTh to cervical spine

CT CAP >> R

Recommendations:

MDT Outcome & Plan
for faslodex /ibrance
ZOMETA EVERY 3NS AFTER DENTAL ASSESSMENT
urgent palliative rth

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د حنان عزت	د احمد نجيب				

MDT Form
[[DOCTYPE:MDT_FORM]]

Fatth Islamic Center

C.T. - M.R.I

U.S. - Duppler - E.C.G. Stress

E.E.G - E.M.G - Mamm - Eye Unit

الجمعية الشرعية

مركز الفتح الإسلامى

أشعة مقطعية بالكمبيوتر - الرنين المغناطيسى
موجات صوتية - دوبلر ملون - رسم قلب بالمجهود
رسم مخ - رسم عضلات - فحص ثدى - وحدة عيون

اسم المريض : ساميه محمد مدبولي التاريخ : ٢٠٢٦/٨/١٩

M.R.I. REPORT

M.R.I. Of The Cervical Spine

Technique:

- *Sagittal T1 & T2 WIs*
- *Axial T_2^* (G.R.E.) WIs*

Findings:

- *Abnormal marrow signal intensity of C2, C3 and C4 vertebral bodies displaying low T1 and low and intermediate T2 signal intensity. Relatively decreased height of C4 vertebral body and rotation of its posterior inferior aspect into the spinal canal with indentation of the subjacent theca and cord.*
- *Paravertebral soft tissue mass suspected intra-spinal epidural mass are seen at C3/C4 level of low T1 and non-homogenous high T2 signal intensity.*
- *The cervical spinal cord shows no masses or areas of abnormal signal intensity.*
- *Spondylotic changes are established with marginal osteophytic formation.*
- *Straightening of the cervical curve is noted.*

Opinion:

- ***Changes seen at C2, C3 and C4 vertebral bodies with cord indentation at C4 level and paravertebral and ? intraspinal epidural soft tissue mass possibly due to neoplastic lesion, for contrast-enhanced M.R.I. study CT scan for the cervical spine.***
- ***Cervical spondylosis.***

YOURS SINCERELY,

Prof. Dr. Mohy El-Hennawy

PROF. OF RADIO-DIAGNOSIS

AL-AZHAR UNIVERSITY

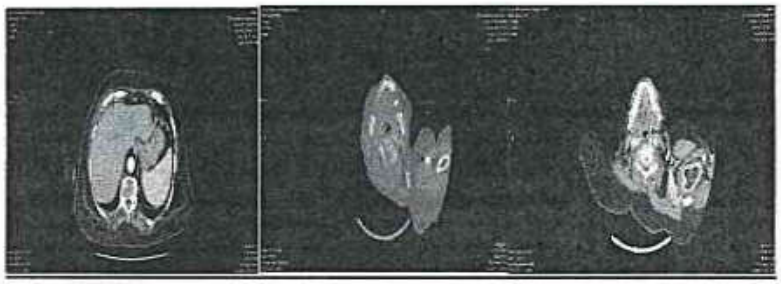
٥٣ شارع ١٠ بالمعادى ت : ٢٣٥٨٧٣٣٢ - ٢٣٥٨٥٨١٩ - ٠١١٠٠١٥١٣٨٠ - ٠١٠٢٨٤٨٣٦٩٥


جمهورية مصر العربية
بطاقة تحقيق الشخصية
ساميه
محمد مدبولى عمر
مساكن رولان-بلوك ٣٠-مدخل ٢
المقطم - القاهره
٢٥٤١٠٢٨٠١٠٣٠٦١
IE9180024


٢٥٤١٠٢٨٠١٠٣٠٦١ ٢٠١٩/١٠
ربة منزل
انثى مسلمة متزوجة
عادل سيد ابراهيم محمد
البطاقة سارية حتى ٢٠٢٦/١٠/١٥


ERM036723444

Medical No. : 11066
Patient's Name: Samia Mohammed Madbouli Omar
Date: 2026-10-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-
CHEST WITHOUT & WITH CONTRAST

- Evident right mastectomy with no evidences of either residual and /or recurrent neoplastic lesions seen herein.
- **Normal left breast.**
- **Multiple variable sized benign-looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 8.5 mm.**
- -Hyperinflated both lungs.
- -Mild subpleural reticulations are seen in the anterior segment of right upper pulmonary lobe.
- -Linear atelectatic bands are seen scattering in the lungs with a ground-glass opacity seen in the anterior segment of left lower pulmonary lobe.
- -No patchy areas of consolidations ,abnormally-enhancing pulmonary nodules or foci of pulmonary calcifications noted.
- -An enhancing pretracheal lymph node is seen and measures about 11mm in diameter.
- -No other sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- -Patent central airways.
- -Bilateral minimal pleural reactions and /or thickenings are seen.
- -No pleural or pericardial effusion.
- **-Border line size of the heart with atherosclerotic changes of its big vessels.**
- -Foci of osteolysis are seen at different levels of cervical spine **with spondylo-degenerative changes scattering in the rest of the scanned spine.**

ABDOMEN AND PELVIS WITHOUT & WITH CONTRAST:

-Average sized liver,measuring about 13.6 cm, showing a diffuse reduced normal parenchymal attenuation and a non-homogenous enhancement pattern.
-Multiple variable sized hypovascular focal lesions in both hepatic lobes and the largest one measures about 28mm in segmewnt 111 of left hepatic lobe.
-No dilated biliary radicles.
-Patent PV.
-Average sized spleen,measuring about 10.1 cm. No focal lesions seen inside.
-Relatively thickened both adrenal glands.
-Normal CT appearance of the pancreas, functioning both kidneys, aorta and IVC.
-No sizable abdominal or pelvic lymphadenopathy.
-No ascites.
-The pelvic structures are unremarkable.

IMPRESSIONS:

Medical No. :	**11066**
Patient's Name:	**Samia Mohammed Madbouli Omar**
Date:	**2026-10-12**
Referred by:	Prof.Dr. no doctor
Hospital:	**Goustave Roussey International Egypt**

- Evident right mastectomy with no evidences of either residual and /or recurrent neoplastic lesions seen herein.
- An enhancing pretracheal lymph node, with multiple variable sized hypovascular focal lesions in both hepatic lobes, relatively thickened both adrenal glands and foci of osteolysis seen at different levels of cervical spine,likely to be metastatic. For close clinical and radiological follow up.
- Mild subpleural reticulations in the anterior segment of right upper pulmonary lobe,likely to be a post therapeutic sequelae.
- Linear atelectatic bands scattering in the lungs with a ground-glass opacity seen in the anterior segment of left lower pulmonary lobe,and bilateral minimal pleural reactions and /or thickenings, likely to be inflammatory reactions.

Prof/Hanan El-Ahwal.

Dr.Hanan Elahwel